

2026 Maternal Immunization Schedule

Vaccines help keep pregnant patients and their growing families healthy.



Endorsed by:

American Academy of Family Physicians (AAFP), American Academy of Pediatrics (AAP), American Academy of Physician Associates (AAPA), American College of Nurse-Midwives (ACNM), Association of Physician Associates in Obstetrics and Gynecology (APAOG), American Pharmacists Association (APhA), Association of Women's Health, Obstetric and Neonatal Nurses (AWHONN), Council of Medical Specialty Societies (CMSS), Infectious Diseases Society of America (IDSA), Infectious Diseases Society for Obstetrics and Gynecology (IDSOG), National Medical Association (NMA), National Association of Nurse Practitioners in Women's Health (NPWH), Society for Maternal-Fetal Medicine (SMFM)

	VACCINE	GIVEN DURING EVERY PREGNANCY	GIVEN TO SPECIFIC GROUPS DURING PREGNANCY	CONTRAINDICATED DURING PREGNANCY	CAN BE INITIATED WHILE BREASTFEEDING AND WHILE POSTPARTUM
ROUTINELY RECOMMENDED VACCINES DURING PREGNANCY (use this table in conjunction with table on page 2)	Inactivated or recombinant influenza	X ¹ Administer at any time of the year, at any gestational age			X ¹
	COVID-19	X ² Administer at any time of the year, at any gestational age			X ²
	Tetanus toxoid, reduced diphtheria toxoid, and acellular pertussis (Tdap)	X ³ Administer at any time of the year, at 27–36 weeks of gestation			X ³
	Maternal RSV (Abrysvo)	X ⁴ Administer seasonally in first eligible pregnancy, at 32 weeks 0 days–36 weeks 6 days of gestation; in subsequent pregnancies, repeat vaccination not indicated and infants should receive a monoclonal antibody			
RECOMMENDED VACCINES BASED ON COMORBIDITIES OR DISEASE RISK FACTORS	Pneumococcal		X ⁵		X ⁵
	Meningococcal conjugate (MenACWY or MenABCWY) and meningococcal serogroup B		X ^{6,7}		X ^{6,7}
	Hepatitis A		X ⁸		X ⁸
	Hepatitis B		X ^{8,9}		X ^{8,9}
	Human papillomavirus (HPV)				X ¹⁰
	Measles-mumps-rubella (MMR)			* ¹¹	X ¹¹
	Varicella			* ¹²	X ¹²

How to Use This Schedule:

- An “X” indicates that the vaccine can be given in this window. An asterisk (*) indicates that the vaccine is contraindicated in pregnancy.
- This schedule was adapted from ACOG’s Committee Statement No. 26, Maternal Immunizations: <https://www.acog.org/clinical/clinical-guidance/committee-statement/articles/2026/02/maternal-immunizations>.
- For additional information and clinical guidance for each vaccine, refer to the references on page 3.

Helpful Information:

- Vaccine information statements: <https://www.immunize.org/vaccines/vis/about-vis/>
- Committee Statement No. 26, Maternal Immunizations: <https://www.acog.org/clinical/clinical-guidance/committee-statement/articles/2026/02/maternal-immunizations>
- ACOG Immunization Tools and Resources: [acog.org/immunizationtools](https://www.acog.org/immunizationtools)
- Patient resources: www.acog.org/vaccine-resources

Summary of Routinely Recommended Maternal Vaccines

Vaccine	Aug	Sep	Oct	Nov	Dec	Jan	Feb	Mar	Apr	May	Jun	Jul
Tdap	Can be administered at any time											
COVID-19		Administer as soon as available	However, it can be administered at any time of the year to eligible individuals									
Influenza		Ideally administer in the early fall	However, it can be administered at any time while the virus is circulating.									
RSV		Administer September through January in most of the continental U.S.*										



Tdap Vaccine

Vaccine Product: Any Tdap vaccine product may be administered.

Schedule: Administer a dose of Tdap, preferably during the early part of gestational weeks 27 through 36, during each pregnancy irrespective of the patient's prior Tdap vaccination history.

For more information, see [Committee Opinion: Update on Immunization and Pregnancy: Tetanus, Diphtheria, and Pertussis Vaccination](#).



COVID-19 Vaccine

Vaccine Product: Any COVID-19 vaccine product may be administered.

Schedule: Vaccination may occur in any trimester, and emphasis should be on vaccine receipt as soon as possible to maximize maternal and fetal health.

For more information, see [Practice Advisory: COVID-19 Vaccination Considerations for Obstetric-Gynecologic Care](#).

These vaccines
can be safely
co-administered.



Influenza Vaccine

Vaccine Product: Only administer inactivated influenza vaccine (IIV) or recombinant influenza vaccine (RIV) products.

Schedule: Administer a dose of IIV or RIV to people who are pregnant during any trimester or will be pregnant during influenza season.

Influenza vaccination should be given before the start of the influenza season, by the end of October, but vaccination at any time during the influenza season is encouraged to ensure protection during the period that virus is circulating in the community.

For more information, see [Practice Advisory: Influenza in Pregnancy: Prevention and Treatment](#).



RSV Vaccine

Vaccine Product: Only administer Pfizer's RSV vaccine (Abrysvo), approved as a one-time dose. Infant monoclonal antibody may be administered to the infant as an alternative to vaccinating during pregnancy and in subsequent pregnancies after a patient has received Abrysvo in a previous pregnancy.

Schedule: Administer a dose of Pfizer's RSV vaccine (Abrysvo) only between 32 weeks 0 days and 36 weeks 6 days of gestation during September through January in most of the continental United States if the patient was not previously vaccinated.

*In jurisdictions with seasonality that differs from most of the continental United States (eg, Alaska, jurisdictions with tropical climates), health care professionals should follow state, local, or territorial guidance on the timing of administration.

For more information, see [Practice Advisory: Maternal Respiratory Syncytial Virus Vaccination](#).

Additional vaccines may be needed based on ...

- Patient's age; health status; risk of exposure; and vaccination history such as hepatitis, meningococcal, and pneumococcal vaccines. Visit ACOG's [Immunization Tools and Resources](#) for more information.

References

1. American College of Obstetricians and Gynecologists. Influenza in pregnancy: prevention and treatment. Practice Advisory. ACOG; 2025. Accessed January 23, 2026. <https://www.acog.org/clinical/clinical-guidance/practice-advisory/articles/2025/08/influenza-in-pregnancy-prevention-and-treatment>
2. American College of Obstetricians and Gynecologists. COVID-19 vaccination considerations for obstetric–gynecologic care. Practice Advisory. ACOG; 2020. Accessed January 23, 2026. <https://www.acog.org/clinical/clinical-guidance/practice-advisory/articles/2020/12/covid-19-vaccination-considerations-for-obstetric-gynecologic-care>
3. Update on immunization and pregnancy: tetanus, diphtheria, and pertussis vaccination. Committee Opinion No. 718. American College of Obstetricians and Gynecologists. *Obstet Gynecol* 2017;130:e153–7. doi: 10.1097/AOG.0000000000002301
4. American College of Obstetricians and Gynecologists. Maternal respiratory syncytial virus vaccination. Practice Advisory. ACOG; 2023. Accessed January 23, 2026. <https://www.acog.org/clinical/clinical-guidance/practice-advisory/articles/2023/09/maternal-respiratory-syncytial-virus-vaccination>
5. Dutra K, Berry H, Lazenby GB. Pneumonia vaccines: indications for use and current safety data in pregnancy. *Am J Perinatol* 2025;42:1809–18. doi: 10.1055/a-2505-5434
6. Folaranmi T, Rubin L, Martin SW, Patel M, MacNeil JR. Use of serogroup B meningococcal vaccines in persons aged ≥10 years at increased risk for serogroup B meningococcal disease: recommendations of the Advisory Committee on Immunization Practices, 2015. Centers for Disease Control (CDC) [published erratum appears in *MMWR Morb Mortal Wkly Rep* 2015;64:806]. *MMWR Morb Mortal Wkly Rep* 2015;64:608–12.
7. Amin AB, Collins JP, Dong X, Leidner AJ, Loehr J, Moser CA, et al. Use of the GSK MenACWY-CRM/MenB-4C pentavalent meningococcal vaccine among persons aged ≥10 years: recommendations of the Advisory Committee on Immunization Practices - United States, 2025. *MMWR Morb Mortal Wkly Rep* 2026;75:7–14. doi: 10.15585/mmwr.mm7501a2
8. Viral hepatitis in pregnancy. Clinical Practice Guideline No. 6. American College of Obstetricians and Gynecologists. *Obstet Gynecol* 2023;142:745–59. doi: 10.1097/AOG.0000000000005300
9. Sandul AL, Rapposelli K, Nyendak M, Kim M. Updated recommendation for universal hepatitis B vaccination in adults aged 19–59 years - United States, 2024. *MMWR Morb Mortal Wkly Rep* 2024;73:1106. doi: 10.15585/mmwr.mm7348a3
10. Human papillomavirus vaccination. ACOG Committee Opinion No. 809. American College of Obstetricians and Gynecologists. *Obstet Gynecol* 2020;136:e15–21. doi: 10.1097/AOG.0000000000004000
11. American College of Obstetricians and Gynecologists. Measles, mumps, rubella (MMR) vaccination and management of obstetric–gynecologic patients during a measles outbreak. Practice Advisory. ACOG; 2024. Accessed January 23, 2026. <https://www.acog.org/clinical/clinical-guidance/practice-advisory/articles/2024/03/management-of-obstetric-gynecologic-patients-during-a-measles-outbreak>
12. Cytomegalovirus, parvovirus B19, varicella zoster, and toxoplasmosis in pregnancy. Practice Bulletin No. 151. American College of Obstetricians and Gynecologists [published erratum appears in *Obstet Gynecol* 2016;127:405]. *Obstet Gynecol* 2015;125:1510–25. doi: 10.1097/01.AOG.0000466430.19823.53

The American College of Obstetricians & Gynecologists has neither solicited nor accepted any commercial involvement in the development of the content of this published product.
Published online on June 10, 2026

Copyright 2026 by the American College of Obstetricians & Gynecologists. All rights reserved. No part of this publication may be reproduced, stored in a retrieval system, posted on the internet, or transmitted, in any format by any means, electronic, mechanical, photocopying, recording, or otherwise, without prior written permission from the publisher.

American College of Obstetricians & Gynecologists
409 12th Street SW, Washington, DC 20024-2188

The American College of Obstetricians & Gynecologists (ACOG) reviews its publications regularly; however, its publications may not reflect the most recent evidence. A reaffirmation date is included in the online version of a document to indicate when it was last reviewed. The current status and any updates of this document can be found on ACOG Clinical at [acog.org](https://www.acog.org). This information is designed as an educational resource to aid clinicians in providing obstetric and gynecologic care, and use of this information is voluntary. This information should not be considered as inclusive of all proper treatments or methods of care or as a statement of the standard of care. It is not intended to substitute for the independent professional judgment of the treating clinician. Variations in practice may be warranted when, in the reasonable judgment of the treating clinician, such course of action is indicated by the condition of the patient, limitations of available resources, or advances in knowledge or technology.

While ACOG makes every effort to present accurate and reliable information, this publication is provided “as is” without any warranty of accuracy, reliability, or otherwise, either express or implied. ACOG does not guarantee, warrant, or endorse the products or services of any firm, organization, or person. Neither ACOG nor its officers, directors, members, employees, or agents will be liable for any loss, damage, or claim with respect to any liabilities, including direct, special, indirect, or consequential damages, incurred in connection with this publication or reliance on the information presented.

The 2026 Maternal Immunization Schedule has been formally endorsed by these medical and health organizations:



American Academy
of Pediatrics



DEDICATED TO THE HEALTH OF ALL CHILDREN®



American Academy of
Physician Associates

